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Name: سهم عبد العزيز عبد الحميد علي National ID: 27901041700385	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

menna Ahmed

Dms

ممنى أحمد

جمهورية مصر العربية
بطاقة تحقيق الشخصية



سها م
عبد العزيز عبد الحميد على ناصف
طبيب
مركز شين الكوم - المنوفية

١٩٧٩/٠٨/٠٤ ٢٧٩٠١٠٤١٧٠٠٣٨٥

123375933

٢٠٢١/٠٨ ٢٧٩٠١٠٤١٧٠٠٣٨٥

حاصلة على دبلوم تجارة

أنثى
مسلمة
متزوجة

وليد فتحي عبد الحميد عبد الجواد سقط
البطاقة سارية حتى ٢٠٢٨/٠٨/٢٧



١٢





قسم الباثولوجي

السن : 44 سنة

اسم المريض : سهام عبد العزيز عبد الحميد علي نام

(2023) (1920 - 2023)

مستوى : 1

تاريخ الاستلام : 2023/07/18

تاريخ التصوير : 2023/07/13

NATURE OF SPECIMEN :

One paraffin block of cancer breast.

GROSS :

One paraffin block: sections immunostained for ER, PR, Her-2/neu, ki67, using supersensitive monoclonal antibodies with En-Vision TM+ (HRP) detection system employing peroxidase after antigen retrieval and blocking Internal peroxidase and using DAB as chromogen using BenchMark GX automated immunostainer.

MICROSCOPY :

Right:

Estrogen receptors: Strong nuclear staining in 40% of tumor cells.

Progesterone receptors: Moderate nuclear staining in 10% of tumor cells.

Her-2/neu: No membranous staining of tumor cells.

Ki67: Positive nuclear staining in 3% of tumor cells.

Left:

Estrogen receptors: Strong nuclear staining in 50% of tumor cells.

Progesterone receptors: Weak nuclear staining in 10% of tumor cells.

Her-2/neu: No membranous staining of tumor cells.

Ki67: Positive nuclear staining in 3% of tumor cells.

DIAGNOSIS :

RIGHT:

POSITIVE ESTROGEN RECEPTORS 7/8.

POSITIVE PROGESTERONE RECEPTORS 4/8.

NEGATIVE HER-2/neu, SCORE 0

Ki67: POSITIVE NUCLEAR STAINING IN 3% OF TUMOR CELLS.

LEFT

POSITIVE ESTROGEN RECEPTORS 7/8.

POSITIVE PROGESTERONE RECEPTORS 3/8.

NEGATIVE HER-2/neu, SCORE 0

Ki67: POSITIVE NUCLEAR STAINING IN 3% OF TUMOR CELLS.

RECOMENDATION :

2023/07/18

د. رانيا لاروق



قسم الباثولوجي

العمر : 44 سنة

(2023) (1920) (2023)

اسم المريض : سهام عبد العزيز عبد الحميد على نام

محل من :

تاريخ الإستلام : 2023/07/18

تاريخ الحضور : 2023/07/10

NATURE OF SPECIMEN :

Right and left breast masses core needle biopsy.

GROSS :

Specimens received in two containers designated as right and left

Right: multiple gray white tissue cores collectively measured 0.5x0.5cm, totally submitted

Left: multiple gray white tissue cores collectively measured 0.7x0.7cm, totally submitted.

MICROSCOPY :

Both cores examined showed invasive lobular carcinoma. Tumor forms tubules, trabeculae and indian file pattern of anaplastic discohesive ductal cells. Tumor cells showed mild anaplasia, moderate tendency to form tubules with infrequent mitosis.

DIAGNOSIS :

Right and left breast masses core needle biopsy;

INVASIVE LOBULAR CARCINOMA, PROVISIONAL CORE GRADE I.

RECOMMENDATION :

أ. م. م. م. م.



Patient Name : س. هاشم عبد العزيز عبد الحميد علي ز.

Date : 19-7-2025

Medical No./ Receipt No 10555

M.R.I Study of the Cervical Spine

Technique

- Sagittal T1 and T2 weighted images.
- Axial T2 weighted images.

Findings:

- A known case of cancer breast shows:
- Multiple osseous infiltrative lesions seen involving CV3, CV4 CV6, DV1 and DV2, largest at DV2 , no significant vertebral collapse, no retropulsion or associated soft tissue components

These lesions shows hyperintense T2/STIR , hypointense T1 signal except that of CV3 that shows hypointense T1/T2 signal (sclerotic)

Regarding patient history of breast cancer, features suggesting osseous metastasis.

- Reduced cervical lordotic curve
- C4/5 focal central disc protrusion indenting ventral subarachnoid space
- Normal dimensions of the cervical canal.
- Normal marrow signal intensity pattern.
- Normal size and signal intensity of the cervical spinal cord.
- Normal cranio-cervical junction.
- No abnormal paraspinal soft tissue masses.

Best regards,

Dr. Waleed Abdel Fattah MD
Mohammed Allam, Msc FEBR

Radiology Department		الكوم
		قسم الأشعة
		التشخيصية
MRI Unit		وحدة الرنين
		المغناطيسي

Patient Name: - **سهام عبد العزيز عبد الحميد علي**
Date: - 23-10-2025

MRI Study of the Brain with contrast

Technique: -

- m
- Axial T1, T2 and FLAIR-weighted images.
- Axial DWI.
- IV contrast

Findings: -

: Known case of breast cancer shows

- An ill-defined irregular lesion involving left Petrus apex, measured about 3.2 x 3.5 cm. the lesion shows iso intense signal in T1, variable (iso to hyperintense) signal intensity in T2, high signal in FLAIR, post contrast heterogenous enhancement. the lesion extends posteriorly towards the mastoid portion of temporal bone anteromedially toward sphenoid bone. The lesion shows partially restricted diffusion.....likely deposits
- The lesion is seen inseparable from the jugular fossa partially invading left the sigmoid venous sinus that showed intraluminal defect extend to venous confluence superiorly and left internal jugular vein inferiorly suggesting tumoral thrombosis
- Right side enlarged upper and lower deep cervical LNs, largest measures 12x17 mm.
- Normal size and shape of the ventricular system.
- Normal white/grey matter interface.
- No gross vascular anomalies.
- No mid line shift.
- No extra-axial lesions.
- Normal posterior fossa.
- Right maxillary sinusitis
- Left mastoiditis

Best regards,
Dr. Waleed Abdelfattah, MD



جامعة المنوفية
مستشفى معهد الأورام
وحدة الطب النووي



Tc^{99m} Bone scintigraphy

سنة ٤٦		اسم المريض	عبد العزيز عبد الحميد ٢
F	التوع	التاريخ	٢٠٢٦/٦/١٣

TECHNIQUE:

Whole body anterior & posterior scans taken 3 hours after I.V injection of (20) mci of the tracer Tc^{99m} (MDP).

Finding:

- Increased radioactive tracer uptake over:
- Both shoulder .
- LT hip joint , head of femur .
- LT knee , L3 .
- Sterno clavicular joint .
- D11 , D10 , D12 (bilateral ribs) .
- Cervical spine .
- Occipital skull bone .

OPENION:

- Bone deposits for clinical and radiological correlation .
- In comparison with previous bone scan (8/2025) showed : stable disease .

Verified by:

Prof/Amira Ebrahim Aldesoky
Assistant Professor of clinical Oncology &
nuclear medicine